Surveillance Coverage, Clinical-Process Anchors, and Workload Constraints in Machine Learning Early Warning for Sepsis: An Operational Evaluation Audit

Alireza Shayan, BSc[1], **Somayeh Afrasiabi**, PhD[1,*]

[1]Department of Computer Science and Engineering & IT,
School of Electrical and Computer Engineering, Shiraz University,
Molla Sadra St., Shiraz 71348-51154, Fars, Iran
Telephone: +98-71-3613-3500 Email: afrasiabi@shirazu.ac.ir

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[*]Corresponding Author: Somayeh Afrasiabi (afrasiabi@shirazu.ac.ir)

Abstract

Objectives: To evaluate how surveillance coverage, clinical-process anchors, and alert workload constraints govern estimated performance and evaluability in sepsis early warning.

Materials and Methods: We evaluated 8 predictive architectures across 8,254 first ICU stays from MIMIC-III MetaVision (2008–2012; 97,700 windows; hourly window-level prevalence 0.694% vs. stay-level sepsis prevalence 4.94%) during hours 19–30. We formalized an operational cascade ($S \to V \to O \to D$), quantified order date coarsening, and evaluated alert frontiers under 12-hour alert suppression.

Results: Among 408 operational Sepsis-3 stays ($S = 1$), 257 (63.0%) developed onset after hour 36 ($V = 0$). Among 151 covered stays ($V = 1$), 100 (66.2%) were first-bin assigned ($T_{\text{susp}} - T_{\text{S3}} \in [24\text{h}, 25\text{h})$), structurally guaranteeing pre-suspicion eligibility; 50 of the remaining 51 were also eligible ($O = 1$: 150/151, 99.3%). Conditional on fixed operational Sepsis-3 timestamps, evaluating opportunity against earliest prescription dates under midnight assignment eliminated all horizon-positive observations (0/151 definite; bounds [0.0%, 31.1%] covered, [0.0%, 11.5%] cohort-wide), preventing discrimination estimation. At 10 alert episodes/100 patient-days (407 emitted alerts), BiMamba SSM and LightGBM each detected 7 distinct patients ($D = 1$), yielding episode PPV 1.72% (7/407, NNE 58.1), covered sensitivity 4.64% (7/151), and population yield 1.72% (7/408).

Discussion: Evaluability and lead-time estimates depend fundamentally on protocol coverage and process-anchor definitions.

Conclusion: Sepsis predictive evaluations must report protocol coverage, distinguish structurally induced eligibility from biological lead time, and benchmark under explicit bedside workload constraints.

Keywords: Sepsis Early Warning, Surveillance Truncation, Anchor Selection, Alert Budget Frontier, State-Space Models.

Lay Summary: Machine learning systems designed to provide early warnings for sepsis in intensive care units are frequently evaluated using historical health records that assume alarms could have alerted doctors hours before treatment began. In this study, we audited 8,254 ICU stays and eight AI architectures to examine how real-world clinical timelines affect early warning accuracy. We found that reported multi-hour advance warning times largely stem from the standard rules used to date sepsis backwards in medical databases rather than earlier detection of illness. Furthermore, when medication records only record calendar dates rather than minute-by-minute administration, determining whether an alarm arrived before treatment becomes highly uncertain. Finally, when alarms are capped to realistic levels (10 alarms per 100 patient-days), AI models successfully identified under 2% of sepsis patients, requiring clinicians to review over 58 alerts for every true case detected. These results highlight the critical need to design and test clinical AI under real-world hospital workflow constraints.

1 Introduction

1.1 Background and Significance

Sepsis remains a leading cause of critical illness, inpatient mortality, and healthcare expenditure globally [1, 2]. Clinical treatment urgency depends critically on presentation: the Surviving Sepsis Campaign international guidelines recommend immediate antimicrobial therapy, ideally within one hour, for possible septic shock or a high likelihood of sepsis, while recommending rapid diagnostic assessment for possible sepsis without shock [3, 4]. To support prompt intervention, the biomedical informatics community has heavily invested in machine learning (ML) models for early warning. Scores of retrospective investigations have reported high discriminative capability, frequently reporting area under the receiver operating characteristic curve (AUROC) values between 0.80 and 0.88 ahead of consensus onset [5]. However, published ICU prediction studies differ substantially in outcome implementation, temporal alignment, and evaluation design; consequently, reported AUROCs should not be interpreted as directly comparable measures of deployment readiness [5, 6]. Prior MIMIC-III analyses have shown that alternative operational onset definitions can change measured performance as much as or more than changing model class [7]. These findings motivate separating reference-event construction from surveillance coverage and alert-policy evaluation. Time-sensitive sepsis benchmarks have also incorporated utility functions that reward and penalize alerts according to their timing [8]. Such approaches reinforce the need to report temporal evaluation rules, although benchmark utility is distinct from measured clinical benefit.

Clinical evaluations have reported heterogeneous performance and outcome associations [9, 10], while clinician-vignette studies have identified discrepancies between operational onset labels and recommended treatment timing [11]. External evaluation of the original Epic Sepsis Model demonstrated that reported performance depended heavily on the evaluation level and timing rules: hospitalization-level and prediction-level workload estimates were not interchangeable, emphasizing the need to report the exact unit of evaluation [9]. Prospective evaluations of early-warning systems such as TREWS have found associations between timely alert confirmation and improved outcomes among selected patients identified prior to antibiotic initiation [10, 12]. However, because provider response was observational rather than randomized, these findings support potential clinical utility without establishing universal causal efficacy [10].

Furthermore, ICU clinicians already encounter substantial alarm

exposure from bedside physiological monitors [13]. In other clinical decision-support settings, repeated alerts and work complexity have been associated with lower alert acceptance, supporting explicit reporting of notification burden [14, 15].

Recent evaluations of sepsis early warning before clinical intervention present divergent findings. Kamran et al. [16] reported a severe decline in Epic model AUROC (0.62 to 0.47) when excluding predictions following clinical recognition. Conversely, Dutta et al. [17] demonstrated preserved discrimination under certain pre-clinical action definitions across nine hospitals. Do et al. [18] further showed that continuous surveillance and alert silencing rules fundamentally alter clinical workload across health systems. This literature indicates that pre-intervention performance does not follow a uniform collapse, but depends on how onset labels, clinical-process anchors, and evaluation units are constructed.

These findings motivate the central question of this study: *Why do estimates of pre-clinical action opportunity and alert performance diverge across evaluation designs?* We formalize an operational evaluation audit framework across four distinct cascade stages ($S \rightarrow V \rightarrow O \rightarrow D$):

1. **Incident Target Sepsis (S):** Study-defined operational Sepsis-3 incidence within the monitored population ($S_i \in \{0, 1\}$).
2. **Surveillance Protocol Coverage (V):** Fixed observation schedules miss episodes occurring outside the evaluated surveillance interval ($V_i \in \{0, 1\}$; 63.0% of incident cases develop onset after hour 36).
3. **Clinical-Process Anchors and Timestamp Uncertainty (O):** Pre-exposure decision opportunities depend on whether anchors are placed at clinical suspicion (T_{susp}) or recorded medication orders ($T_{\text{Rx,start}}$). High pre-suspicion opportunity (150/151, 99.3%) is largely an algebraic artifact of the discrete 24h SOFA lookback window in first-bin assigned cases (100/151, 66.2%; $G_i \in [24\text{h}, 25\text{h})$), whereas calendar-day coarsening of prescription orders eliminates all definite opportunity (0/151, bounds [0.0%, 31.1%]).
4. **Emitted-Alert Workload Constraints (D):** Operational alert budgets (5, 10, 20 episodes/100 patient-days) with 12-hour alert suppression bound real-world sensitivity and positive predictive value ($D_i \in \{0, 1\}$), while exploratory outcome-stratified hybrid sets ($\mathcal{R}_0^{\text{hybrid}}, \mathcal{R}_2^{\text{hybrid}}$) demonstrate that discrimination stability reflects control-window censoring rather than a uniform prospective policy.

2 Materials and Methods

2.1 Study Design and Cohort Definition

We conducted a retrospective evaluation with sequential prediction replay using the MIMIC-III critical care database (v1.4) [21], evaluating transportability across clinical information systems from CareVue (2001–2008 development: $N = 12,964$ candidates; 10,371 train, 2,593 validation; 1,687 incident sepsis) to MetaVision (2008–2012 evaluation replay: $N = 9,732$ candidates; 1,886 sepsis). The base cohort comprised first ICU stays of adult patients (≥ 18 years) with total ICU length of stay LOS ≥ 30h. While conditioning on LOS ≥ 30h ensures that included patients remain hospitalized throughout mid-stay surveillance (hours 19–30), at surveillance entry ($t = 19$h) it retrospectively conditions on 11 hours of future ICU survival and stay. Stays with community-onset sepsis prior to or upon ICU arrival ($T_{S3} \leq 0$h) were excluded as prevalent, and stays developing consensus Sepsis-3 during the baseline pre-surveillance interval ($0 < T_{S3} \leq 19$h, $N = 1,478$) were excluded prior to surveillance entry, yielding the evaluated prospective-replay test cohort of 8,254 unique stays (408 incident sepsis, 7,846 controls). Beyond the primary MetaVision evaluation, historical sensitivity analyses included $N = 3,581$ incident sepsis stays across CareVue (1,687), MetaVision (1,886), and 8 transitional stays bridging both database systems ($1,687 + 1,886 + 8 = 3,581$). Complete cohort derivation schemas, exclusion criteria, feature dictionaries, missing-data rules, model architectures, and executable Sepsis-3 definitions are detailed in the Supplementary Methods Package.

Surveillance was evaluated under a prespecified second-day ICU surveillance protocol: predictions were generated at 12 scheduled hourly decision bins $[t-1, t]$ for $t \in \{19, 20, \ldots, 30\}$ hours post-admission. In prospective streaming deployment, patients would enter surveillance at hour 19 and be administratively right-censored at discharge or death. The test partition comprised 8,254 unique patient ICU stays (97,700 evaluated windows, 4,070.8 patient-days of surveillance volume). Stays were censored at Sepsis-3 onset ($N = 110$ stays, 623 windows) or upon observation-availability cessation prior to hour 30 ($N = 180$ control stays, 725 windows, triggered by cessation of charted vital sign telemetry in the EHR). Reporting was organized in accordance with TRIPOD+AI and STROBE recommendations [22, 23].

2.2 Outcome Definition and Operational Clocks

Sepsis was defined according to a study-specific operational adaptation of Sepsis-3 criteria [1, 2]: an acute increase in Sequential Organ Failure Assessment (SOFA) score ≥ 2 points over baseline. Baseline SOFA ($\text{SOFA}_{\text{base}}$) was defined per consensus operational implementations as the minimum total SOFA score observed in the 24-hour lookback window preceding the suspicion anchor ($t \in [T_{\text{susp}} - 24\text{h}, T_{\text{susp}}]$, assumed 0 if unmeasured). Sepsis-3 onset was identified when total SOFA increased by ≥ 2 points above $\text{SOFA}_{\text{base}}$ within the clinical search window $[T_{\text{susp}}-24\text{h}, T_{\text{susp}}+12\text{h}]$ relative to suspected infection (defined as blood culture specimen collection paired with systemic antimicrobial prescription orders; detailed in Supplementary Section S1.5). Because $\text{SOFA}_{\text{base}}$ is determined retrospectively across the candidate lookback, for first-bin episodes near $T_{\text{susp}} - 24$h, the minimum can occur after assigned onset if organ dysfunction subsequently resolves; this identifies the first qualifying bin relative to a retrospective search-window minimum rather than an observed increase from a pre-ICU baseline. In the MIMIC-III database, antimicrobial timing derives from recorded prescription scheduling dates (`prescriptions.startdate`), which represent medication order records rather than electronically verified bedside administration timestamps. While MetaVision records minute-resolved intravenous infusions in `inputevents_mv`, we intentionally audit the date-coarsened `prescriptions` table without constructing an ad-hoc composite anchor: order records reflect standard benchmark practice, preserve compatibility with CareVue cohorts, capture oral and ward orders omitted from continuous infusion tables, and isolate the specific timestamp uncertainty intrinsic to retrospective order records.

To resolve temporal precision and event ambiguity without conflating orders, intended start dates, and physical administrations, we track six operational milestones (with ICU admission as the reference origin $t = 0$, followed by five operational event clocks; Figure 1):

- $T_{\text{admit},i}$: ICU admission timestamp ($t = 0$, reference origin).
- $T_{\text{Rx,start},i}$: Earliest documented systemic antimicrobial order start date across the hospital stay (`prescriptions.startdate`, calendar date coarsened to midnight 00:00:00; captures admission empiric or prophylactic orders).
- $T_{\text{abx,pair},i}$: Systemic antimicrobial order date participating in the qualifying infection-suspicion pair with blood culture ([−24h, +72h] window per Seymour et al. [2]).

- $T_{\text{culture,pair},i}$: Blood culture specimen collection timestamp (`microbiologyevents.charttime`, minute-level precision).
- $T_{\text{susp},i}$: Operational infection-suspicion anchor, defined per Seymour et al. [2] as $\min(T_{\text{abx,pair}}, T_{\text{culture,pair}})$.
- $T_{\text{S3},i}$: Operationally assigned Sepsis-3 onset timestamp.

When blood culture precedes antimicrobial prescription ($T_{\text{culture,pair}} < T_{\text{abx,pair}}$), $T_{\text{susp}} = T_{\text{culture,pair}} \neq T_{\text{abx,pair}}$. Let $h = 1$ hour and define the dimensionless lookback coordinate $x_i = (T_{\text{susp},i} - 24\text{h} - T_{\text{admit},i})/h$. The search window evaluates discrete hourly bins relative to admission, starting at integer index $\texttt{baseline_start_h}_i = \lfloor x_i \rfloor$ and ending at $\texttt{window_end_h}_i = \lfloor (T_{\text{susp},i} + 12\text{h} - T_{\text{admit},i})/h \rfloor$. When $\text{frac}(x_i) > 0$, discrete flooring places $\lfloor x_i \rfloor$ up to < 1h earlier than the continuous boundary $T_{\text{susp}} - 24\text{h}$. Because the upper boundary bin is floored, evaluated hours never exceed $T_{\text{susp}} + 12\text{h}$, guaranteeing $T_{\text{susp},i} - T_{\text{S3},i} \geq -12.0\text{h}$ (empirical minimum: −11.2h). For episodes where acute deterioration $\text{SOFA}(T_{\text{S3},i}) - \text{SOFA}_{\text{base},i} \geq 2$ is first observed at the earliest search bin ($N = 274/408$, 67.2% of all incident sepsis; and 100/151, 66.2% of surveillance-covered stays)—occurring when organ dysfunction relative to the lookback minimum $\text{SOFA}_{\text{base}}$ is already established at the earliest evaluated bin (where baseline minimum across $[T_{\text{susp}} - 24\text{h}, T_{\text{susp}}]$ may occur after the initial bin if dysfunction subsequently improves)—onset is assigned as $T_{\text{S3},i} = T_{\text{admit},i} + h\lfloor x_i \rfloor$, yielding the operational lead gap:

$$G_i = T_{\text{susp},i} - T_{\text{S3},i} = 24\text{h} + h\ \text{frac}(x_i) \tag{1}$$

Because $0 \leq \text{frac}(x_i) < 1$ (dimensionless), first-bin-assigned episodes satisfy $24\text{h} \leq G_i < 25\text{h}$, with an analytical supremum of 25h (the reported 24.99h is the observed empirical maximum). In these first-bin episodes (100/151, 66.2% of surveillance-covered stays), pre-suspicion opportunity ($t < T_{\text{susp}}$) is algebraically guaranteed for any decision time evaluated prior to onset ($t < T_{\text{S3}} \approx T_{\text{susp}} - 24\text{h}$), and the 2-hour buffer is trivially satisfied (100/100 eligible under both $\delta = 0\text{h}$ and $\delta = 2\text{h}$). For the remaining 51 non-first-bin covered stays (33.8%), the median lead gap was 19.3h (IQR: 7.1–22.0h; 45 of 51 exhibiting $T_{\text{susp}} > T_{\text{S3}}$). Because evaluated early-warning decisions occur up to 6h prior to onset ($t < T_{\text{S3}}$), pre-suspicion opportunity holds for 50 of the 51 non-first-bin stays under $\delta = 0\text{h}$ (1 ineligible stay, detailed in Supplementary Audit Table) and 49 of 51 under $\delta = 2\text{h}$ (2 ineligible stays). Across all 151 covered stays, pre-suspicion eligibility is thus 100 + 50 = 150, corresponding to 150/151 (99.3%) under $\delta = 0\text{h}$ and 100 + 49 = 149, corresponding to 149/151 (98.7%) under $\delta = 2\text{h}$. Thus, high pre-suspicion opportunity largely reflects this operational retrospective lookback convention rather than independent empirical evidence of unprompted biological warning. Across the full 408 cohort, the median lead gap was +24.2h (IQR: 22.8h to 24.6h; range: −11.2h to +24.99h).

2.3 Operational Cascade and Outcome-Stratified Hybrid Risk Sets

Let C denote the evaluated test cohort of unique ICU stays ($N = 8,254$), and let $\mathcal{T}_i \subseteq \{19, 20, \ldots, 30\}$ denote the set of hourly decision bins evaluated for stay i. For patients who never develop operational Sepsis-3 during hospitalization, we set $T_{\text{S3},i} = \infty$.

The full evaluated early-warning risk set is defined as:

$$\mathcal{R} = \left\{(i,t) : i \in C,\ t \in \mathcal{T}_i,\ t < T_{\text{S3},i}\right\} \tag{2}$$

The binary early-warning prediction target at anticipation horizon $H = 6\text{h}$ is:

$$Y_i(t;H) = \mathbf{1}\left\{t < T_{\text{S3},i} \leq t + H\right\} \tag{3}$$

In the canonical unconstrained risk set ($\mathcal{R}_{\text{canonical}} = \mathcal{R}$), this definition yields exactly 97,700 evaluated hourly windows, comprising 678 horizon-positive observations ($Y = 1$) and 97,022 negative observations ($Y = 0$, prevalence $\pi = 0.694\%$).

The four stages of the operational cascade ($S \to V \to O \to D$) are formalized via stay-level indicators: $S_i = \mathbf{1}\{T_{\text{S3},i} < \infty\}$, $V_i = \mathbf{1}\{\exists t \in \mathcal{T}_i : Y_i(t;6\text{h}) = 1\}$, $O_i(\delta) = \mathbf{1}\{\exists t \in \mathcal{T}_i : Y_i(t;6\text{h}) = 1 \wedge t + \delta \leq C_i\}$, and $D_i(q,\delta) = \mathbf{1}\{i \in \mathcal{S}_{\text{alerted}}(K) \wedge Y_i(t_{\text{first},i};6\text{h}) = 1 \wedge t_{\text{first},i} + \delta \leq C_i\}$. To evaluate restricted decision windows, we define outcome-stratified hybrid risk sets relative to reference clock C_i:

$$\mathcal{R}_\delta^{\text{hybrid}} = \left\{(i,t) \in \mathcal{R} : t + \delta \leq C_i\right\} \tag{4}$$

where $C_i = T_{\text{susp},i} = \min(T_{\text{culture,pair},i}, T_{\text{abx,pair},i})$ for incident sepsis stays ($S_i = 1$; determined by $T_{\text{abx,pair}}$ in 399 stays and culture charttime in 9 stays), and $C_i = T_{\text{Rx,start},i}$ for controls ($S_i = 0$) if antimicrobials were ordered ($C_i = \infty$ otherwise). Exactly 2,287 control stays had $T_{\text{Rx,start}} \leq 19\text{h}$, censoring all their windows ($W_{\text{fully excluded}} = 27,388$). Among 83 later-prescription control stays ($T_{\text{Rx,start}} > 19\text{h}$), 21 intersected surveillance ($t \in [19, 30]$), censoring 113 windows at $\delta = 0\text{h}$ ($65,926 = 93,427 - 27,388 - 113$), while 62 occurred after hour 30; 49 additional windows were removed by the 2-hour margin ($65,877 = 65,926 - 49$), including 7 windows from stays with $T_{\text{Rx,start}} \in (30, 32]\text{h}$ where $t \in \{29, 30\}$ satisfies $t \leq T_{\text{Rx,start}}$ but violates $t + 2\text{h} \leq T_{\text{Rx,start}}$. Together with 4,266 sepsis case windows (674 positive, 3,592 negative; 7 censored vs. canonical; eligible covered stays retain $O_i = 1$ via remaining positive bins), $\mathcal{R}_0^{\text{hybrid}}$ evaluates 70,192 windows across 5,967 stays (prevalence 0.960%). Moving to $\delta = 2\text{h}$ removes 63 total windows ($70,192 - 63 = 70,129$): 49 control windows and 14 sepsis case windows (11 positive [674 → 663] and 3 negative [3,592 → 3,589] violating $t + 2\text{h} \leq T_{\text{susp},i}$, leaving 4,252 case windows; prevalence 0.945%).

Importantly, cases and controls used different anchors; differences from canonical discrimination reflect both evaluated subsets and outcome-dependent selection, not a uniform policy. Under declared midnight assignment, all 408 sepsis stays had documented prescription start dates prior to hour 3.83 of ICU admission ($T_{\text{Rx,start}} \leq 3.83\text{h}$, median −12.6h) because every patient received empiric or prophylactic antimicrobial orders on or before admission day, whereas qualifying acute suspicion pairs ($T_{\text{abx,pair}}$) occurred later (median +50.7h). Consequently, all 4,273 case windows were excluded from earliest-prescription sets as post-prescription ($t \geq 19\text{h} > T_{\text{Rx,start}}$: 678 positive, 511 baseline negative, and 3,084 late-onset negative windows). Surviving observations in $\mathcal{R}_0^{\text{Rx}}$ (5,559 stays, 65,926 negative windows, 0 positive) and $\mathcal{R}_2^{\text{Rx}}$ (65,877 negative windows; $N_+^{\text{Rx}} = 0$; prevalence strictly 0.000%) derive solely from controls. Discrimination was thus mathematically unestimable: AUROC is undefined and AUPRC uninformative.

2.4 Alert-Budget Operating Frontiers and Refractory Policy

To quantify simulated emitted-alert burden, we evaluated fixed alert budgets $q \in \{5.0, 10.0, 20.0\}$ emitted alert episodes per 100 surveillance patient-days, targeting $K = \text{round}(q \cdot E/100)$ alert episodes across total surveillance exposure $E = \sum_i h_i/24 = 4,070.8$ patient-days (yielding $K = 204, 407, 814$). Because surveillance spans a 12-hour interval ($t \in \{19, \ldots, 30\}$), a 12-hour refractory suppression rule guarantees at most one alert episode per patient stay ($N_{\text{episodes},i} \leq 1$), as any alert emitted at $t \in [19, 30]$ silences subsequent alerts past hour 30. Consequently, simulating chronological refractory alerts is algebraically equivalent to selecting the top K patient stays ranked by their peak surveillance score $\max_{t \in \mathcal{T}_i} s_i(t)$. We report three operational metrics:

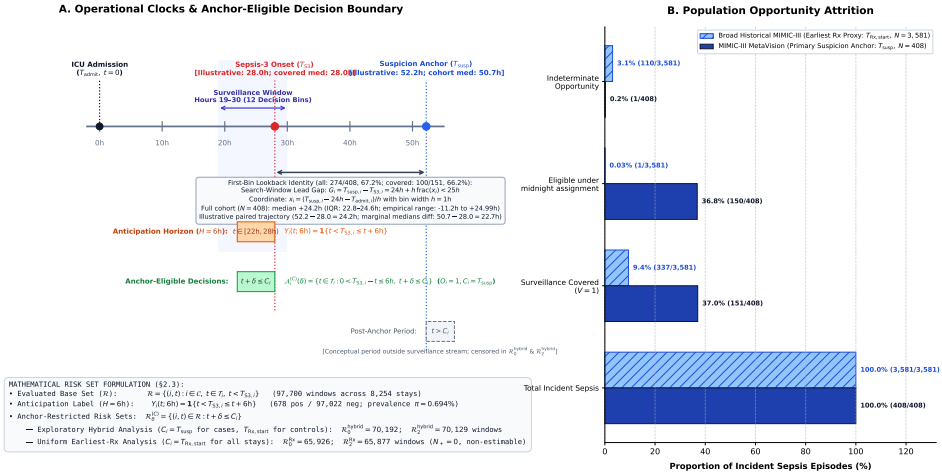


Figure 1: Operational Clocks and Anchor-Dependent Evaluation Eligibility in the MIMIC-III MetaVision Cohort. The timeline is schematic; displayed population summaries do not represent a single paired patient trajectory. Eligibility relative to a retrospective suspicion anchor does not establish absence of prior antimicrobial exposure. Panel A depicts the clinical timeline for an ICU-incident sepsis stay, illustrating the discrete search-window lead $G_i = 24\text{h} + h\ \mathrm{frac}(x_i) < 25\text{h}$ for first-bin-assigned episodes ($N = 274/408$, 67.2%; median +24.2h; empirical max 24.99h), the anticipation horizon $t \in [22\text{h}, 28\text{h})$, anchor-eligible decision times $\mathcal{A}_i^{(C)}(\delta)$, and the conceptual post-anchor period. Actual model evaluations occur at discrete scored timestamps $t \in \mathcal{T}_i$. Panel B displays population opportunity attrition comparing Broad Historical MIMIC-III (earliest prescription proxy $T_{\text{Rx,start}}$, $N = 3,581$) and MIMIC-III MetaVision (suspicion anchor T_{susp}, $N = 408$), reporting exact stay counts beside each opportunity tier under declared midnight assignment; this comparison reflects concurrent differences in both cohort selection and anchor definition rather than isolating an anchor effect alone. *Alt-text:* Two-panel diagram displaying schematic timeline clocks and formal risk-set definitions in Panel A, and horizontal bar charts comparing surveillance coverage and treatment-precedence opportunity in Panel B.

Table 1: Cohort Progression and Denominator Accounting Ledger. Denominators traced from the base test cohort through surveillance eligibility, sepsis onset timing, prediction horizons, and pre-treatment actionability. *Note:* Rows 1–3b report cohort shares; rows 4–6 report window-level positive prevalence (π).

Progression Stage / Cohort Partition	Stays (N)	Evaluated Windows	Surveillance Volume	Prev. / Share	Clinical Definition & Measurement Convention
1. Base Prospective-Replay Test Cohort	8,254	97,700 / 99,048 sched.	4,070.8 pt-days	100.0%	Adult first ICU stays ≥ 30h; 1,348 censored windows (623 onset, 725 cessation)
2. Non-Sepsis Control ICU Stays	7,846	93,427 neg. windows	3,892.8 pt-days	95.06%	Stays without operational Sepsis-3 during hospitalization
3. Operational Sepsis-3 Stays ($S = 1$)	408	4,273 (678 pos, 3,595 neg)	178.0 pt-days	4.94%	Blood culture + antibiotic + acute ΔSOFA ≥ 2; full cohort median onset 49.0h
3a. Late-Onset Sepsis (Onset > 36h)	257	3,084 neg. windows	128.5 pt-days	63.0% of septic	Onset > 36h (median: 69.0h); anticipation horizon $[T_{S3} - 6, T_{S3}]$ outside $[19, 30]$ ($V = 0$)
3b. Surveillance-Covered ($V = 1$)	151	1,189 (678 pos, 511 neg)	49.5 pt-days	37.0% of septic	Onset in [20h, 36h] (median: 28.0h); anticipation horizon intersects window ($V = 1$, earliest uncensored integer onset bin is 20h)
4. Canonical Sepsis-3 Risk Set ($H = 6$h)	8,254	97,700 (678 pos / 97,022 neg)	4,070.8 pt-days	0.694%	Full unconstrained risk set ($t \in \mathcal{T}_i$, $t < T_{S3,i}$)
5. Outcome-Stratified Hybrid Set $\mathcal{R}_0^{\text{hybrid}}$ ($\delta = 0$h)	5,967	70,192 (674 pos / 69,518 neg)	2,924.7 pt-days	0.960%	Censoring post-anchor ($t \le C_i$; T_{susp} cases, $T_{\text{Rx,start}}$ controls); 150/151 covered eligible
6. Outcome-Stratified Hybrid Set $\mathcal{R}_2^{\text{hybrid}}$ ($\delta = 2$h)	5,967	70,129 (663 pos / 69,466 neg)	2,922.0 pt-days	0.945%	2-hour proxy-timing margin ($t + 2\text{h} \le C_i$); 149/151 covered eligible

Episode PPV $= N_{\text{det}}/K$, Covered Sensitivity $= N_{\text{det}}/151$, and Population Yield $= N_{\text{det}}/408$, where timely detections $N_{\text{det}}(q, \delta)$ are defined as:

$$N_{\text{det}}(q,\delta) = \sum_{i \in \mathcal{S}_{\text{alerted}}(K)} \mathbf{1}\{S_i = 1 \wedge Y_i(t_{\text{first},i}; 6\text{h}) = 1 \wedge t_{\text{first},i} + \delta \le C_i\} \tag{5}$$

Candidate alert selection is performed across the full canonical stream $\mathcal{R}$. Selecting patients by peak score $\max_t s_i(t)$ represents a retrospective batch threshold-solving equivalence; detection timing is adjudicated chronologically at the first threshold crossing: $t_{\text{first},i} = \min\{t \in \mathcal{T}_i : \text{Elig}_i(t; \tau, \mathcal{B}) = 1\}$, where $\text{Elig}_i(t; \tau, \mathcal{B}) = \mathbf{1}\{i \in \mathcal{S}_{\text{alerted}}(K) \wedge s_i(t) \ge \tau\}$, with operating score threshold $\tau(K) = \min_{j \in \mathcal{S}_{\text{alerted}}(K)} \max_{t \in \mathcal{T}_j} s_j(t)$ given by the peak score of the K-th ranked patient replica under deterministic prioritization rule $\mathcal{B}$: $(-\max_t s_i(t),\ \text{ICUSTAY_ID},\ \text{copy_idx})$ (copy_idx = 0 for point estimates). Because surveillance-covered sepsis cases are censored at onset (mean 7.87 evaluated bins, vs 10.47 across all 408 sepsis stays) while controls are observed across a mean of 11.91 bins (12 bins for uncensored controls; 180 control stays lose 725 windows through observation cessation), differential stay durations can introduce multiple-testing duration bias under peak-score aggregation ($\max_t s_i(t)$), as characterized by Do et al. [18].

2.5 Predictive Models and Statistical Inference

We evaluated 8 models across three paradigms: (1) State-Space Models (BiMamba SSM) [24, 25]; (2) Deep Sequence Transformers (Mask-Aware Clinical Transformer and Baseline Unmasked Transformer); (3) Tabular and Ensemble Baselines (Random Forest, XGBoost, LightGBM, L2 Logistic Regression, and Multi-Model Stacking Ensemble). The 21 features comprised 6 vitals (heart rate, systolic, diastolic, and mean arterial pressure, respiratory rate, SpO_2) and 15 laboratory/clinical variables (WBC, platelets, creatinine, BUN, total bilirubin, lactate, glucose, sodium, potassium, bicarbonate, hematocrit, PaO_2, FiO_2, urine output, GCS). Missing values were forward-filled (up to 4h for vitals, 24h for labs); remaining unobserved intervals were median-imputed. At surveillance entry ($t = 19$h), lookback $[t - 20\text{h}, t) = [-1\text{h}, 19\text{h})$ intersects pre-admission bin $[-1\text{h}, 0\text{h})$; for $t \ge 20$h, lookbacks lie entirely post-admission ($[0\text{h}, 30\text{h})$). Charted inputs strictly precede decision time t. Tabular models received the flattened $20 \times 21 = 420$-dimensional vector of raw hourly bins. Sequence models processed length-$L = 20$ normalized series; feature-indexed binary masks ($\mathbf{m}_{i,t} \in \{0,1\}^{21}$) were concatenated (42-dimensional channels) for the Mask-Aware Transformer, with $[-1\text{h}, 0\text{h})$ zero-masked after standardization. Deep models used weighted binary cross-entropy with hidden dimension 64 across 2 layers using AdamW; tree ensembles used 300 estimators (depth 6, learning rate 0.05). The Stacking Ensemble combined out-of-fold cross-validated predictions from LightGBM, XGBoost, Random Forest,

BiMamba SSM, and Clinical Transformer via an L2 logistic meta-learner. BiMamba SSM mean-pooled hidden states with linear $O(L)$ scan complexity ($L = 20$). We did not benchmark ΔSOFA or bedside heuristics (qSOFA, NEWS2, MEWS): ΔSOFA is endogenous to the Sepsis-3 outcome definition, while integer bedside scores have coarse step-function distributions with pervasive ties that resist continuous threshold tuning under fixed alert budgets without arbitrary tie-breaking. Our evaluation restricted its scope to continuous multivariate machine learning architectures, and does not evaluate bedside clinical scores.

Uncertainty was quantified using patient-clustered paired bootstrapping ($B = 1,000$ resamples). For Procedure B (Empirical Burden-Matched Bootstrap), exposure $E_b = \frac{1}{24}\sum_{i \in b} h_i$ (where $h_i = |\mathcal{T}_i|$ is the number of evaluated windows for stay i) and target $K_b = \text{round}(q \cdot E_b/100)$ were dynamically recomputed in each resample. NNE confidence intervals were derived by exact reciprocal transformation from unrounded PPV_b: $\text{NNE}_{\text{low}} = 100/\text{PPV}_{\text{upper}}$ and $\text{NNE}_{\text{high}} = 100/\text{PPV}_{\text{low}}$ ($+\infty$ when $\text{PPV}_{\text{low}} = 0$).

3 Results

3.1 Cohort Progression and Denominator Accounting

Table 1 traces all 8,254 ICU admissions through surveillance eligibility, onset timing, lookback horizons, and pre-treatment actionability. Across all 408 operational Sepsis-3 stays, median onset was 49.0h post-admission. Exactly 257 stays (63.0%; median onset: 69.0h) had clinical onset after hour 36. Under the specified observation schedule and 6-hour horizon, these 257 stays had no evaluated horizon-positive decision time ($V = 0$). The remaining 151 stays (37.0%; median onset: 28.0h) had active protocol surveillance coverage ($V = 1$). Of the 151 covered stays, 150 had at least one horizon-positive decision time at or before the selected infection-suspicion anchor (99.3% within-covered opportunity; 36.8% of total incident sepsis under $\delta = 0$h; and 149 stays, 98.7% under $\delta = 2$h).

3.2 Outcome-Stratified Hybrid Discrimination Analysis

Table 2 presents discrimination across Canonical Sepsis-3, Hybrid Set ($\mathcal{R}_0^{\text{hybrid}}$), and Hybrid Set with 2h Buffer ($\mathcal{R}_2^{\text{hybrid}}$). Discrimination changed little in the outcome-stratified hybrid analysis (BiMamba SSM: AUROC 0.6326 canonical vs. 0.6433 on $\mathcal{R}_0^{\text{hybrid}}$, paired ΔAUROC = +0.0107, 95% CI: +0.0060 to +0.0175; Stacking Ensemble: 0.6284 vs. 0.6351, paired ΔAUROC = +0.0067). Because cases and controls were restricted using different anchors, this finding does not establish preserved discrimination under a uniform pre-suspicion or pre-prescription policy. The increase in raw AUPRC (BiMamba: 1.209% to 1.656%) was mechanically driven by control-window censoring, which raised baseline positive prevalence by 1.384× (from $\pi = 0.694\%$ to 0.960%). When evaluated as prevalence-normalized lift (AUPRC/π), discriminative gain over baseline remains essentially flat (1.742× canonical vs. 1.725× on $\mathcal{R}_0^{\text{hybrid}}$ for BiMamba, and 1.738× vs. 1.688× for the Stacking Ensemble), consistent with a substantial contribution from prevalence and evaluated-subset changes; these comparisons do not isolate changes in ranking performance. These results are directionally consistent with the order-based sensitivity findings of Dutta et al. [17].

3.3 Observed Burden–Detection Trade-Off Under This Protocol

Table 3 and Figure 2 demonstrate the observed trade-off under prespecified alert budgets: bedside detection yield is constrained by alert-rate limits. Alerts were selected across the full canonical surveillance stream (4,070.8 patient-days; 97,700 windows across 8,254 test stays); C_i was used only retrospectively to adjudicate advance-warning timing ($t_{\text{first},i} + \delta \leq T_{\text{susp},i}$ for sepsis cases), rather than excluding observations prior to alert selection. Retaining control stays with prior antimicrobial orders in the alert pool simulates unconstrained clinical screening; in live practice, suppressing alerts on patients already receiving broad-spectrum antimicrobials would be an operational deployment option that prevents treated controls from consuming alert capacity.

At an emitted workload of 10 alert episodes per 100 patient-days ($K = 407$ emitted episodes across 4,070.8 patient-days):

- Across all 8 models, episode-level PPV ranged from 0.98% to 1.72%, corresponding to 58.1 to 101.8 simulated evaluations per detected patient (NNE).
- BiMamba SSM and LightGBM each detected **7 distinct patients** (PPV = 1.72%, 95% CI: 0.49%–2.95%; NNE = 58.1, 95% CI: 33.9–203.5; covered sensitivity 4.64%; population yield 1.72%).
- Stacking Ensemble, XGBoost, and Random Forest each detected **6 distinct patients** (PPV = 1.47%, NNE = 67.8).
- Of the 407 emitted alerts for BiMamba SSM, 54 fired on sepsis stays (7 timely detections within $(0, 6]$h of onset; 47 premature alerts emitted > 6h prior to onset) and 353 fired on control stays. Crucially, 71 of these control alerts (17.4% of all 407 alerts; 20.1% of control alerts) fired on pre-treated controls ($T_{\text{Rx,start}} \leq 19$h), while 277 alerts (68.1% of all alerts) fired on untreated controls, and 5 alerts on controls with prescriptions after hour 30. Across models, pre-treated controls consumed 17.4% (BiMamba SSM, Stacking Ensemble; 71/407) to 31.4% (LightGBM; 128/407) of all emitted alerts. With 7 true timely detections ($N_{\text{false}} = 407 - 7 = 400$ false alerts), retrospectively removing alerts on pre-treated controls removes 17.8% (71/400) and 32.0% (128/400) of false alerts for BiMamba SSM and LightGBM, respectively (and 20.1% [71/353] of control alerts for BiMamba SSM). Under a fixed threshold τ, this reduces raw alert volume; under a fixed budget $K = 407$, it reallocates these 71–128 alert slots to untreated candidates. These counts describe retrospective alert occupancy by prescription-proxy-defined control subgroups; a prospective treatment-based suppression rule would apply to all patients and require verified active administration (e.g., continuous IV infusion logs) rather than midnight-coarsened orders ($T_{\text{Rx,start}} \leq 19$h), which would otherwise silence all 408 sepsis stays. Its real-time effects on alert volume, timely detection, and safety were not prospectively evaluated.
- In this observed evaluation, the count of horizon-correct emitted alerts and alerts additionally satisfying the two-hour pre-suspicion margin ($t_{\text{first},i} + 2\text{h} \leq T_{\text{susp},i}$) were identical across all models (7 of 7 for BiMamba SSM; $N_{\text{det}} = N_{\text{det,pre-susp}}$). These detections occurred before the suspicion anchor (T_{susp}), not necessarily before earliest antimicrobial prescribing ($T_{\text{Rx,start}}$).
- Model rankings varied across budgets: at 20 episodes/100 pt-days, XGBoost detected 15 patients (NNE = 54.3) and Mask-Aware Transformer detected 14 patients (NNE = 58.1).

Table 2: Benchmark Discrimination Across Outcome-Stratified Hybrid Risk Sets (Held-Out Point Estimates and Clustered 95% CIs). Point estimates calculated directly on held-out test data ($B = 0$); 95% CIs derived from $B = 1,000$ patient-clustered bootstrap resamples. Evaluated on hybrid risk sets $\mathcal{R}_0^{\text{hybrid}}$ and $\mathcal{R}_2^{\text{hybrid}}$ censoring cases at infection-suspicion anchor T_{susp} and controls at earliest prescription $T_{\text{Rx,start}}$. Sepsis cases and controls were restricted using different clinical-process anchors; differences from canonical discrimination reflect both evaluated observations and outcome-specific selection, and do not represent a uniform prospective eligibility policy.

Model Architecture / Metric	Canonical Sepsis-3 (H = 6h)	Hybrid $\mathcal{R}_0^{\text{hybrid}}$ (δ = 0h)	Hybrid $\mathcal{R}_2^{\text{hybrid}}$ (δ = 2h)	Paired ΔAUROC ($\mathcal{R}_0^{\text{hybrid}}$ − Canon)
BiMamba SSM	0.6326 (0.5872–0.6733)	**0.6433** (0.5998–0.6835)	**0.6447** (0.6002–0.6860)	**+0.0107** (+0.0060 to +0.0175)
AUPRC	*1.209% (0.892–1.769)*	***1.656%** (1.235–2.329)*	***1.647%** (1.220–2.331)*	—
Multi-Model Stacking Ensemble	0.6284 (0.5829–0.6699)	0.6351 (0.5897–0.6771)	0.6370 (0.5913–0.6799)	+0.0067 (+0.0021 to +0.0137)
AUPRC	*1.206% (0.882–1.748)*	*1.621% (1.207–2.263)*	*1.613% (1.198–2.255)*	—
Random Forest	0.6108 (0.5643–0.6535)	0.6143 (0.5684–0.6575)	0.6173 (0.5712–0.6612)	+0.0034 (-0.0008 to +0.0101)
AUPRC	*1.080% (0.821–1.434)*	*1.470% (1.138–1.920)*	*1.461% (1.120–1.916)*	—
Mask-Aware Clinical Transformer	0.5942 (0.5467–0.6410)	0.5961 (0.5490–0.6422)	0.5954 (0.5480–0.6426)	+0.0020 (-0.0024 to +0.0083)
AUPRC	*1.013% (0.766–1.303)*	*1.382% (1.048–1.760)*	*1.365% (1.032–1.752)*	—
Tabular GBDT (XGBoost)	0.5670 (0.5208–0.6102)	0.5643 (0.5186–0.6082)	0.5652 (0.5192–0.6099)	-0.0027 (-0.0070 to +0.0038)
AUPRC	*0.977% (0.739–1.357)*	*1.320% (1.008–1.817)*	*1.313% (0.998–1.811)*	—
Tabular GBDT (LightGBM)	0.5613 (0.5161–0.6026)	0.5587 (0.5139–0.6010)	0.5598 (0.5145–0.6033)	-0.0026 (-0.0070 to +0.0038)
AUPRC	*0.893% (0.694–1.195)*	*1.222% (0.957–1.629)*	*1.214% (0.949–1.630)*	—
Logistic Regression (L2)	0.5593 (0.5097–0.6048)	0.5654 (0.5157–0.6107)	0.5657 (0.5150–0.6127)	+0.0061 (+0.0023 to +0.0116)
AUPRC	*0.939% (0.716–1.218)*	*1.337% (1.012–1.740)*	*1.325% (1.002–1.731)*	—
Clinical Transformer (Baseline)	0.5239 (0.4808–0.5653)	0.5296 (0.4851–0.5712)	0.5286 (0.4834–0.5705)	+0.0057 (+0.0019 to +0.0110)
AUPRC	*0.808% (0.609–1.381)*	*1.147% (0.857–1.904)*	*1.131% (0.845–1.894)*	—

*Paired differences (ΔAUROC) were calculated from unrounded model predictions prior to display rounding.

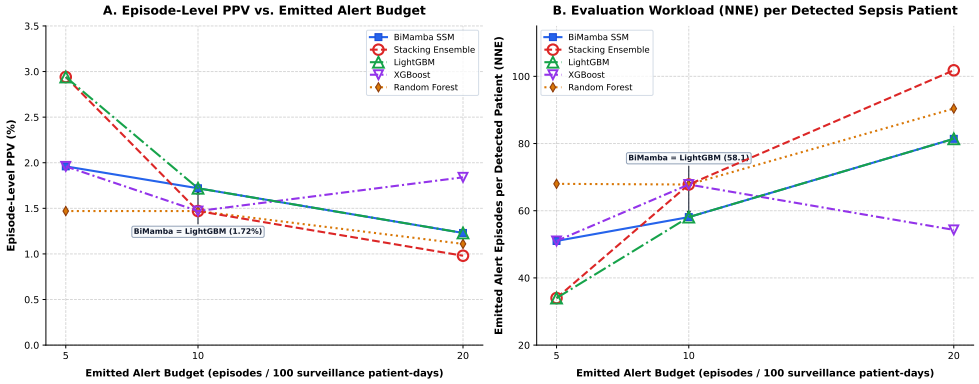


Figure 2: Operational Alert-Budget Performance Frontier. Panel A displays episode-level positive predictive value (PPV %) across prespecified alert budgets. Panel B displays the Number Needed to Evaluate (NNE) per detected patient under a 12-hour alert suppression rule. Lines connect discrete prespecified operating budgets (5, 10, and 20 emitted episodes per 100 patient-days) for a representative 5-model subset; complete 8-model results, bootstrap confidence intervals, and boundary-degenerate notes are reported in Table 3. BiMamba SSM and LightGBM have coincident operating points at budgets of 10 and 20 episodes per 100 surveillance patient-days. *Alt-text:* Two-panel plot showing PPV (Panel A) and evaluation workload per detected patient (Panel B) across alert budgets of 5, 10, and 20 episodes/100 patient-days for 5 models under 12h alert suppression.

3.4 Two-Axis Subgroup and Timestamp Uncertainty Matrix

Table 4 presents the cross-tabulation of Sepsis Onset Timing against Treatment Proxy Timing. Because MIMIC-III prescription start dates (`prescriptions.startdate`) lack time of day, order initiation is evaluated both under declared point assignment (midnight a_i) and under calendar-day intervals $[a_i, b_i)$ = [midnight, next midnight). For stay i, let minute-resolved culture charttime be c_i and the coarsened prescription interval be $[a_i, b_i)$. The transformed suspicion anchor interval is $[L_i, U_i)$, where $L_i = \min(a_i, c_i)$ and $U_i = \min(b_i, c_i)$. A stay has definite opportunity ($O_i^{\text{def}} = 1$) if there exists an evaluated horizon-positive decision bin $t \in \mathcal{T}_i$ satisfying $t + \delta \le L_i$ ($\forall C \in [a_i, b_i), t + \delta \le \min(C, c_i)$); it has possible opportunity ($O_i^{\text{poss}} = 1$) if there exists $t \in \mathcal{T}_i$ such that $t + \delta < U_i$ ($\exists C \in [a_i, b_i) : t + \delta \le \min(C, c_i)$). Because b_i is excluded from $[a_i, b_i)$, strict inequality $t + \delta < U_i$ is required for non-empty feasible realizations. Stays with $O_i^{\text{def}} = 0$ but $O_i^{\text{poss}} = 1$ are indeterminate ($O_i^{\text{indet}} = 1$). All indicators strictly satisfy $O_i^{\text{def}} \le O_i^{\text{point}} \le O_i^{\text{poss}}$ and $O_i^{\text{poss}} = O_i^{\text{def}} + O_i^{\text{indet}}$, with $O_i^{\text{def}} \equiv O_i^{\text{point}}$ by construction because midnight point assignment $a_i = \inf[a_i, b_i)$. Importantly, these opportunity bounds are strictly conditional on fixed operational Sepsis-3 timestamps and surveillance coverage; they do not propagate counterfactual antibiotic timings through the upstream phenotype (which could alter qualifying pairs, baseline SOFA, onset, or coverage).

Opportunity depended strongly on the selected clinical-process anchor. Under the infection-suspicion anchor (T_{susp}), 150 of 151 covered patients (99.3% within-covered; 36.8% of total incident sepsis) had decision bins at or before T_{susp} under declared midnight point assignment a_i. Because $a_i \le T_{\text{susp},i}$ across $[a_i, b_i)$, these 150 stays are definitely eligible throughout the calendar-day interval ($O_i^{\text{def}} = 150$), with only 1 indeterminate stay ($O_i^{\text{indet}} = 1$), bounding opportunity between 36.8% and 37.0% ($[150, 151]/408$; $[99.3\%, 100.0\%]$ within-covered). In contrast, under the earliest hospital prescription-start proxy ($T_{\text{Rx,start}}$ where $L_i = a_i, U_i = b_i$), exactly 0 covered stays had definite opportunity under midnight point-assignment ($O_i^{\text{def}} = 0, O_i^{\text{point}} = 0$), with 47 indeterminate stays ($O_i^{\text{indet}} = 47$), establishing bounds $[0.0\%, 11.5\%]$ ($[0, 47]/408$; $[0.0\%, 31.1\%]$ within-covered). Because no horizon-positive cases remained before point-assigned earliest prescriptions, this task was not evaluable for discrimination under midnight point assignment. In broad historical MIMIC-III ($N = 3,581$), intervals generate 1 definitely eligible stay, 110 indeterminate stays, and 111 possibly eligible stays ($[1, 111]/337 = [0.30\%, 32.94\%]$ within-covered; whole-cohort bounds $[0.03\%, 3.10\%]$ $[[1, 111]/3,581]$). The 150/151 result under T_{susp} does not establish that patients were unexposed to antimicrobials or that models predicted prior to first administration. Equally, zero definite opportunity under $T_{\text{Rx,start}}$ reflects date coarsening, not verified bedside exposure.

3.5 Cumulative Population Coverage Decomposition

Figure 3 illustrates the four-stage cumulative population coverage decomposition ($S \to V \to O \to D$): $\frac{|V=1|}{|S=1|} \times \frac{|O=1|}{|V=1|} \times \frac{|D=1|}{|O=1|} =$

Table 3: Operational Burden-Matched Alert Frontiers Under 12h Refractory Suppression. Evaluated under Procedure B with exposure recomputation and first-emitted-alert timing verification.

Budget	Model Architecture	Cutoff (τ)	Alerts (K)	Episode PPV % [95% CI]	Det (N)	Sens. Cov %	Yield %	NNE [95% CI]
5.0 ep / 100 pt-d	Stacking Ensemble	0.0145	204	2.94 (0.98–5.88)%	6	3.97 (1.31–7.65)%	1.47 (0.49–2.86)%	34.0 (17.0–102.0)
	LightGBM	0.0117	204	2.94 (0.98–5.39)%	6	3.97 (1.26–7.10)%	1.47 (0.47–2.61)%	34.0 (18.5–102.0)
	BiMamba SSM	0.0187	204	1.96 (0.49–4.45)%	4	2.65 (0.65–6.20)%	0.98 (0.24–2.31)%	51.0 (22.5–204.0)
	XGBoost	0.0122	204	1.96 (0.49–3.94)%	4	2.65 (0.63–5.63)%	0.98 (0.23–2.09)%	51.0 (25.4–204.0)
	Random Forest	0.0177	204	1.47 (0.00–3.45)%	3	1.99 (0.00–4.49)%	0.74 (0.00–1.70)%	68.0 (29.0–∞)
	Clinical Transformer	0.0101	204	0.98 (0.00–2.46)%	2	1.32 (0.00–3.50)%	0.49 (0.00–1.27)%	102.0 (40.6–∞)
	Logistic Regression	0.0103	204	0.98 (0.00–2.94)%	2	1.32 (0.00–3.85)%	0.49 (0.00–1.45)%	102.0 (34.0–∞)
	Mask-Aware Transformer	0.0120	204	0.00 (0.00–0.00)%*	0	0.00 (0.00–0.00)%*	0.00 (0.00–0.00)%*	Undefined (∞)
10.0 ep / 100 pt-d	BiMamba SSM	0.0165	407	1.72 (0.49–2.95)%	7	4.64 (1.33–8.16)%	1.72 (0.49–2.93)%	58.1 (33.9–203.5)
	LightGBM	0.0110	407	1.72 (0.49–2.95)%	7	4.64 (1.38–7.74)%	1.72 (0.49–2.89)%	58.1 (33.9–203.5)
	Stacking Ensemble	0.0131	407	1.47 (0.25–2.46)%	6	3.97 (0.70–6.98)%	1.47 (0.26–2.54)%	67.8 (40.7–407.0)
	XGBoost	0.0114	407	1.47 (0.49–2.94)%	6	3.97 (1.32–7.65)%	1.47 (0.49–2.85)%	67.8 (34.0–203.5)
	Random Forest	0.0158	407	1.47 (0.49–2.70)%	6	3.97 (1.26–7.29)%	1.47 (0.47–2.70)%	67.8 (37.0–204.0)
	Mask-Aware Transformer	0.0110	407	1.23 (0.25–2.46)%	5	3.31 (0.74–6.67)%	1.23 (0.26–2.48)%	81.4 (40.7–407.0)
	Clinical Transformer	0.0094	407	1.23 (0.25–2.46)%	5	3.31 (0.68–6.38)%	1.23 (0.25–2.37)%	81.4 (40.7–407.0)
	Logistic Regression	0.0095	407	0.98 (0.25–1.97)%	4	2.65 (0.59–5.51)%	0.98 (0.23–2.07)%	101.8 (50.8–408.0)
20.0 ep / 100 pt-d	XGBoost	0.0105	814	1.84 (0.98–2.70)%	15	9.93 (5.26–14.47)%	3.68 (1.87–5.29)%	54.3 (37.0–101.9)
	Mask-Aware Transformer	0.0099	814	1.72 (0.74–2.58)%	14	9.27 (4.54–13.77)%	3.43 (1.59–5.19)%	58.1 (38.8–135.7)
	Logistic Regression	0.0088	814	1.35 (0.49–2.09)%	11	7.28 (2.74–11.11)%	2.70 (0.99–4.18)%	74.0 (47.8–203.7)
	BiMamba SSM	0.0142	814	1.23 (0.49–2.09)%	10	6.62 (3.07–10.89)%	2.45 (1.05–4.10)%	81.4 (47.9–203.2)
	LightGBM	0.0105	814	1.23 (0.61–2.09)%	10	6.62 (3.11–11.04)%	2.45 (1.15–4.10)%	81.4 (47.9–163.0)
	Random Forest	0.0131	814	1.11 (0.49–1.85)%	9	5.96 (2.70–9.68)%	2.21 (0.96–3.69)%	90.4 (54.2–203.7)
	Stacking Ensemble	0.0115	814	0.98 (0.37–1.72)%	8	5.30 (2.04–9.15)%	1.96 (0.74–3.43)%	101.8 (58.1–271.3)
	Clinical Transformer	0.0088	814	0.86 (0.25–1.48)%	7	4.64 (1.51–8.18)%	1.72 (0.52–3.04)%	116.3 (67.8–407.0)

*At zero observed detections, the empirical percentile-bootstrap interval was degenerate at zero. This does not establish zero population detection probability. NNE intervals were derived by exact reciprocal transformation from unrounded bootstrap $\text{PPV}_b = 100 \cdot N_{\text{det},b}/K_b$ under dynamically recomputed exposure E_b and budget $K_b = \text{round}(q \cdot E_b/100)$. Covered sensitivity and population yield denominators ($N = 151$ and $N = 408$) remain fixed across resamples. In this observed sample, horizon-correct detections and detections satisfying the two-hour pre-suspicion proxy-timing margin ($t_{\text{first},i} + 2\text{h} \leq T_{\text{susp},i}$) were identical across all models and operating points.

Table 4: Two-Axis Subgroup and Timestamp Uncertainty Matrix. Opportunity bounds conditional on fixed operational Sepsis-3 timestamps and surveillance coverage. *Note:* Subgroup Bounds denote $[O^{\text{def}}, O^{\text{poss}}]/\text{Total}(N)$ within each onset stratum. For All Incident Sepsis-3, within-covered bounds ($[O^{\text{def}}, O^{\text{poss}}]/\text{Covered}[V = 1]$) are $[150, 151]/151 = [99.3\%, 100.0\%]$ for MetaVision T_{susp}, $[0, 47]/151 = [0.0\%, 31.1\%]$ for MetaVision Earliest Rx, and $[1, 111]/337 = [0.30\%, 32.94\%]$ for Broad Historical MIMIC-III.

Cohort & Timestamp Source	Sepsis Onset Category	Total (N)	Covered ($V = 1$)	Def. Elig (O^{def})	Point (O^{point})	Indet (O^{indet})	Poss (O^{poss})	Subgroup Bounds	Contribution to Cohort
MetaVision Prospective-Replay	Early Incident (≤ 24h)	47	47 (100.0%)	47	47	0	47	[100.0%, 100.0%]	[47, 47] / 408 (11.5%)
(Primary Benchmark: Suspicion Anchor T_{susp})	Intermediate (24–48h)	156	104 (66.7%)	103	103	1	104	[66.0%, 66.7%]	[103, 104] / 408 (25.2–25.5%)
	Late Incident (> 48h)	205	0 (0.0%)	0	0	0	0	[0.0%, 0.0%]	0 / 408 (0.0%)
	All Incident Sepsis-3	**408**	**151 (37.0%)**	**150**	**150**	**1**	**151**	**[36.8%, 37.0%]**	**[150, 151] / 408 (36.8–37.0%)**
MetaVision Sensitivity: Earliest Rx	Early Incident (≤ 24h)	47	47 (100.0%)	0	0	38	38	[0.0%, 80.9%]	[0, 38] / 408 (0.0–9.3%)
(Pre-Exposure Sensitivity: Earliest Rx $T_{\text{Rx,start}}$)	Intermediate (24–48h)	156	104 (66.7%)	0	0	9	9	[0.0%, 5.8%]	[0, 9] / 408 (0.0–2.2%)
	Late Incident (> 48h)	205	0 (0.0%)	0	0	0	0	[0.0%, 0.0%]	0 / 408 (0.0%)
	All Incident Sepsis-3	**408**	**151 (37.0%)**	**0**	**0**	**47**	**47**	**[0.0%, 11.5%]**	**[0, 47] / 408 (0.0–11.5%)**
Broad Historical MIMIC-III	Early Incident (≤ 24h)	2,641	102 (3.9%)	0	0	74	74	[0.00%, 2.80%]	[0, 74] / 3,581 (0.00–2.07%)
(Date-Level Proxy: Earliest Rx $T_{\text{Rx,start}}$)	Intermediate (24–48h)	361	235 (65.1%)	1	1	36	37	[0.28%, 10.25%]	[1, 37] / 3,581 (0.03–1.03%)
	Late Incident (> 48h)	579	0 (0.0%)	0	0	0	0	[0.00%, 0.00%]	[0, 0] / 3,581 (0.00%)
	All Incident Sepsis-3	**3,581**	**337 (9.4%)**	**1**	**1**	**110**	**111**	**[0.03%, 3.10%]**	**[1, 111] / 3,581 (0.03–3.10%)**

$$\frac{151}{408} \times \frac{150}{151} \times \frac{7}{150} = \frac{7}{408} = 1.72\%.$$

1. **Surveillance Truncation** ($151/408 = 37.0\%$)**:** 63.0% of sepsis patients develop onset after hour 36, outside the pre-specified second-day surveillance protocol.
2. **Suspicion-Anchor Eligibility** ($150/151 = 99.3\%$)**:** Of the 151 covered stays, 150 had at least one horizon-positive decision time at or before the retrospective suspicion anchor (T_{susp}) [149 of 151 (98.7%) under a 2-hour pre-suspicion margin]. Together, surveillance coverage and suspicion-anchor eligibility impose a theoretical population ceiling of $150/408 = 36.76\%$.
3. **Model-Alert Policy Detection** ($7/150 = 4.67\%$)**:** At 10 alert episodes/100 patient-days ($K = 407$), an oracle ranking policy could theoretically detect all 150 eligible patients (PPV $150/407 = 36.86\%$). Under the empirical score ranking of BiMamba SSM with 12-hour alert suppression, the model detects 7 distinct patients (4.67% of anchor-eligible stays; 4.64% of covered stays [7/151]; 1.72% whole-population yield [7/408]; NNE = 58.1). Low empirical detection yield is thus driven by model discrimination and alert suppression, not a mathematical ceiling of the budget.

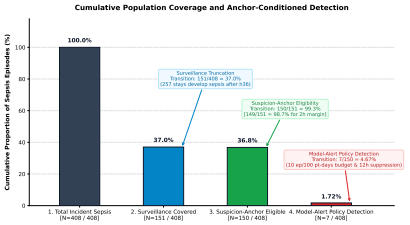


Figure 3: Cumulative Population Coverage and Anchor-Conditioned Detection. Stepwise attrition from 408 incident sepsis episodes through surveillance protocol coverage, retrospective suspicion-anchor eligibility, and model-alert policy detection. *Alt-text:* Four-stage bar chart showing patient opportunity attrition from total incident sepsis (408 patients) to surveillance coverage (151 patients, 37.0%), suspicion-anchor eligibility (150 patients, 36.8%), and timely model-alert policy detection (7 patients, 1.72%).

4 Discussion

This investigation characterizes the operational trade-offs governing machine learning early warning for sepsis in ICU surveillance:

1. **Outcome-Dependent Anchor Selection Governs Evaluability:** The canonical surveillance analysis quantified population coverage and emitted-alert yield. A secondary outcome-stratified hybrid restriction produced small changes in discrimination, but did not represent a uniform prospective eligibility policy. Under a common earliest-prescription-start proxy and declared midnight assignment, no eligible horizon-positive observations remained, preventing discrimination estimation for that restricted task.
2. **Detection Yield Is Constrained by Workload and Prevalence:** AUROC values near 0.64 are modest. At 10 emitted episodes per 100 evaluation-bin patient-days, PPV ranged from 0.98% to 1.72%, corresponding to 58.1–101.8 simulated evaluations per detected patient (NNE); population-level timely yield was 1.72% under the prespecified surveillance schedule.
3. **Surveillance Protocol Coverage Explains Unreached Cases:** Under the evaluated second-day protocol, 63.0% of incident cases develop onset after the monitoring window terminates, preventing multi-hour warning.

4.1 Distinguishing Care-Process Forecasting from Sepsis Anticipation

Forecasting antibiotic initiation and anticipating sepsis represent distinct prediction tasks. When benchmarks predict antibiotic

initiation across all patients, they evaluate care-process forecasting (predicting clinician ordering behavior, including prophylactic prescribing in non-septic patients) rather than physiological sepsis anticipation. For the sepsis-anticipation estimand considered here, evaluations retain the physiological sepsis outcome while evaluating eligibility relative to clinical-process anchors; replacing the disease outcome with treatment receipt evaluates an entirely different estimand.

Clinician-vignette studies address when clinicians judge treatment appropriate [11]; our analysis addresses prediction performance relative to operational EHR proxies. These are complementary questions, and proxy precedence does not establish clinical appropriateness. Furthermore, while changes under exploratory outcome-stratified hybrid restriction were small in this cohort, differences in outcome construction, population selection, timestamp precision, and clinical-action definition preclude direct replication of Dutta et al.'s order-based sensitivity analysis [17]. Clinical utility ultimately depends on alert acceptance, clinician review, and actionable treatment responses [12, 14]. Our analysis characterizes operational opportunities and simulated evaluation burden (NNE); it does not evaluate clinician adoption, cognitive fatigue, or therapeutic efficacy directly [15].

4.2 Limitations

The analysis estimates operating characteristics in a selected ICU cohort rather than clinical effectiveness. Conditioning on LOS $\geq$ 30h at surveillance entry (t = 19h) retrospectively conditions on 11 hours of future ICU survival and stay; furthermore, excluding patients developing sepsis prior to surveillance entry ($0 < T_{S3} \leq 19$h, $N = 1,478$) restricts findings to mid-stay surveillance rather than early triage. Additionally, differential surveillance lengths between right-censored covered sepsis cases (mean 7.87 hourly bins, vs 10.47 across all sepsis stays) and controls (mean 11.91 bins across all controls; 12 bins for uncensored controls) can introduce multiple-testing duration bias under peak-score aggregation ($\max_t s_i(t)$), as characterized by Do et al. [18]. Burden-matched cutoffs were derived retrospectively from evaluated score trajectories, and identifier-based selection was required for tied boundary scores; the frontier does not represent a prospectively locked policy or guarantee a specified alert rate in online streaming deployment.

Temporal precision also constrains pre-treatment opportunity assessment: while MetaVision records minute-resolved intravenous infusions in `inputevents_mv`, legacy sepsis benchmarks standardly extract `prescriptions.startdate` to capture ward and oral orders. Among the 151 surveillance-covered sepsis stays, this prescription proxy yields 0 definitely eligible, 47 indeterminate, and 104 definitely ineligible stays ($[0.0\%, 31.1\%]$ within-covered opportunity bounds; $[0.0\%, 11.5\%]$ cohort-wide). Because we assigned date-coarsened values to midnight for the primary point analysis (a_i = 00:00:00), pre-treatment opportunity cannot be definitively confirmed without minute-level electronic medication administration records (eMAR). The study did not measure clinician response, review duration, treatment changes, alarm fatigue, or patient outcomes; NNE therefore represents a simulated evaluation count rather than measured clinical workload. Alarm-reduction interventions may suppress both nonactionable and clinically important signals; our short evaluation interval cannot establish the safety or optimal duration of a refractory policy during sustained surveillance [15]. Finally, our evaluation benchmarked continuous multivariate ML models and did not include discrete integer bedside scores (qSOFA, NEWS2), whose tie-dominated step-function distributions resist continuous alert-budget tuning, restricting our benchmark scope rather than asserting ML superiority. Our cohorts originate from a single medical center (BIDMC) across EHR transitions (CareVue to MetaVision); multi-center validation across diverse health systems remains essential.

5 Conclusion

In this restricted ICU cohort, observed detection yield depended on surveillance coverage, baseline discrimination, and emitted-alert policies. Evaluation feasibility was highly anchor-dependent: horizon-positive observations were retained before the selected suspicion anchor, but none remained before the earliest prescription-start proxy under midnight assignment, precluding discrimination estimation for that task. Early-warning evaluations must transparently identify outcome-dependent evaluation rules, report anchor-dependent evaluability, and benchmark under explicit workload constraints.

Ethics Approval and Consent to Participate: This study was conducted using de-identified data from the MIMIC-III (v1.4) database, collected at Beth Israel Deaconess Medical Center (BIDMC). Data collection was approved by the Institutional Review Boards of BIDMC and MIT. The data were de-identified in accordance with HIPAA Safe Harbor standards; informed consent was waived by reviewing IRBs. Access was granted under PhysioNet Credentialed DUA. **Competing Interests:** The authors declare no competing interests. **Funding:** None. **Authors' Contributions:** AS conceived the study, developed the framework, implemented pipelines, performed experiments, and drafted the manuscript. SA provided academic supervision, contributed to methodological design, critically revised the manuscript, and approved the final draft.

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Code and Data Availability: Model architectures and frozen weight tensors are archived under commit `b0932ca`; evaluation code under commit `1292103`; denominator recomputation under commit `311cea3`; audit reconciliation and submission release packaging under commit `8f10435` (release tag `v1.0.1-freeze`). Preprocessing and evaluation pipelines are available under MIT License at `https://github.com/alireza/Sepsis-Onset-Timing-Paradox`. De-identified data are available from PhysioNet under MIMIC-III DUA.

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